



Observational Study

Lived experiences of Jordanian mothers of children with cancer: A descriptive qualitative study

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Abstract

BACKGROUND

Caring for children with cancer deeply affects mothers' daily lives, emotions, and family roles; however, to date, little has been learned about these experiences in the context of Jordan.

AIM

To examine the lived experiences of Jordanian mothers of children with cancer, with a focus on their coping mechanisms, support systems, and challenges.

METHODS

This descriptive qualitative study involved the use of semi-structured interviews with 20 mothers of children visiting a pediatric hospital in Jordan. Thematic analysis was employed, guided by the transactional model of stress and coping and the theory of spiritual coping, to analyze the resulting data.

RESULTS

Three major themes were revealed. The idea of a transformative coping journey included several subthemes, namely shifting priorities, holding back tears to appear strong, learning important lessons, and reconstructing futures while delaying personal dreams. The second emergent theme, perceived challenges,

included subthemes around the emotional irritability of the sick child; the impact on family life and daily routines; and preparation of the home environment for children after recovery. Finally, the theme of support systems included subthemes regarding emotional and practical support from extended family and the use of mothers' deep beliefs in destiny and God's will as key sources of strength and endurance.

CONCLUSION

Developing a broad range of support services in the Jordanian context to assist parents whose children have been diagnosed with cancer is essential. Possible focuses for such programs could include the provision of counselling services, support groups, and educational resources to help parents more effectively manage stress, thereby enhancing their emotional well-being.

Key Words: Lived experience; Mothers; Cancer; Jordan; Descriptive qualitative study; Pediatric cancers

Core Tip: This qualitative study was a descriptive one that investigated the life experiences of Jordanian mothers with cancer children. Three themes were identified after in-depth interviews, namely transformative coping journey, support systems, and perceived challenges. Mothers coped by switching priorities and keeping emotions under wraps to seem strong and meaning-driven by religious faith in God. The most important part in battling stress was emotional and practical family support, as well as spiritual beliefs. The research indicates the necessity of culturally sensitive psychosocial and educational programs that would help mothers go through the complicated environment of pediatric cancer care in Jordan.

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INTRODUCTION

Childhood cancer is a major cause of mortality among children. Its incidence has been seen to be increasing worldwide in recent years, with an estimated 400000 children under the age of 19 now diagnosed every year[1]. Most children in high-income countries see remission of such cancers with proper treatment; however, fewer than 30% of cases are properly treated in those countries with low and middle incomes[1,2].

The effects of childhood cancer on families have been examined in a range of countries and cultures. Such effects include issues with early treatment and a lack of compliance with medical care due to perceptions of economic, emotional, and spiritual expenses[3]. Families may feel a sense of shame, fear, or confusion when their children become ill, and parents often suffer economic burdens, including medical bills, missing income due to taking time off work to care for the child, and related expenses[4].

The treatment of cancer in its initial stages is often radical, involving multiple hospitalization sessions and travel for treatment, which may interfere with family routines and duties[3]. Due to the additional caregiving burden such treatment imposes, parents of cancer patients also tend to experience reduced quality of life[5] and heightened effects of stress, anxiety, and desperation[4]. The Transactional Model of Stress and Coping guided the coping process used by some mothers, in that they process the stressful situations they are in and the resources available to them to determine their coping reactions. This framework is therefore useful in interpreting the ways participants maneuver through their experiences and adjust over time[6].

While childhood cancer has been researched globally, little is known concerning the ways in which Jordanian mothers more specifically cope with the practical and emotional challenges associated with this condition. The conclusions of previous related studies in Jordan have been homogeneous in that they have focused only on the spiritual beliefs of parents, leaving a need for more detailed studies to address aspects such as the practical, social, and emotional elements of care delivery in Jordan[7,8]. The literature on lived experiences of parents of children with cancer also remains relatively sparse globally, with no such studies being performed specifically in Jordan. Based on this, the purpose of this research is to explore the lived experience of mothers of children with cancer in Jordan to identify and potentially explain their coping strategies, difficulties, and perceived social support.

The specific research questions were: (1) What are the coping mechanisms used by Jordanian mothers of children with cancer; (2) What are the challenges reported by Jordanian mothers of children with cancer; and (3) What support systems help Jordanian mothers of children with cancer cope during their caregiving journeys?

MATERIALS AND METHODS

Study design

This study employed a descriptive qualitative methodology to explore the subjective experiences of mothers caring for children with cancer in Jordan.

Setting

This study was conducted at a referral pediatric hospital in Amman, Jordan. This hospital treats children with a variety of cancers, including hematological conditions.

Participant recruitment

Purposive sampling was applied to the recruitment of participants; such recruitment continued until data saturation was reached. The eligibility criteria required the participants to be mothers of children aged 0 to 18 years who had been hospitalized in any of the pediatric oncology units of the study hospital. Eligible mothers were those who had children with any type of cancer, whether newly diagnosed, currently undergoing active treatment, or undergoing follow-up cancer treatment.

Following receipt of ethical approval, the principal investigator (PI) liaised with the nursing and medical administration of the various pediatric oncology units to discuss the purpose of the study. The chief nurses initially identified potential participants in accordance with the eligibility criteria, then eligible mothers were given invitation letters and information sheets. The PI then contacted each of these potential participants by telephone to discuss the study further, answer any participant questions, seek verbal consent for participation, and schedule convenient times and locations for the interviews.

Data collection and management

The parents of children with cancer identified by the chief nurses were approached to attend semi structured interviews that were recorded in audio format; each was asked to give their consent before their interview. The semi-structured interview questions were formulated through critical examination of past qualitative research examining the experiences of parents with children with cancer. A literature review was used to determine the critical areas most frequently reported on by parents, including their emotional reactions, coping tactics, family life disruptions, caregiving difficulties, and supporting factors. The development of the interview guide was guided by the transactional model of stress and coping by Folkman[6], which helped to develop ways to explore how mothers perceived, coped, and made sense of the ill health of their children. However, the interview guide development was also informed by the theory of spiritual and religious coping by Pargament[9], which helped to develop understanding of how the mothers perceived, coped, and made sense of the illness of their child in their specific religious context. The questions were designed to be open-ended, to give the mothers the opportunity to narrate their experiences in their own words and to let them raise those issues that they considered the most significant[6,10]. After every interview, the researcher summarized the points that were discussed and the participants confirmed the content.

Data analysis

Transcription of the interviews was done verbatim, without artificial intelligence, using a professional transcription service. The transcripts thus obtained were stored safely in a password-protected system that could only be accessed by the principal researcher, to ensure confidentiality. In order to preserve the original meaning, the transcripts were not translated into English prior to analysis.

Thematic analysis[10] was performed on the transcripts. The PI, an Arabic native speaker, studied the data thoroughly before inputting the transcripts into the chosen analysis software (NVivo 11) in order to enhance the process of organizing, processing, and presenting the data. Data collection and analysis were performed simultaneously, offering the PI the opportunity to assess the data several times and interpret it as part of an organic process[10]. Open coding was thus used to develop codes to highlight potentially meaningful data segments[11]. Relationships were identified through sorting the different codes into possible themes, after which the PI evaluated the themes and grouped them based on source. The sources of information used thus included the researcher, verbatim statements by participants and the prior literature[11]. Finally, the emergent themes were cross-linked with the data from the transcripts and the coded records to ensure that the themes reflected the participants' narratives correctly. The chosen excerpts were coded and reviewed by both the PI and a senior qualitative researcher, with a consensus achieved by discussion.

All the interviews were analyzed in the native Arabic languages used to maintain both cultural meaning and linguistic overtones. Analysis of selected quotations translated into English was then conducted to assist in reporting. While some risk of loss of subtle meanings arises from even such limited translation exists, the strategy reduced its possible impact on the overall interpretation and main messages of the study.

Trustworthiness

To ensure the trustworthiness of the research, all participants received complete transcripts of their recorded conversations that they were then asked to review for accuracy. The PI, who has several years of clinical experience in a pediatric medical department in one of the hospitals in Jordan, has also participated in some clinical teaching in this hospital: While she has not been directly employed in the field of pediatric oncology, her background thus offered extensive experience in pediatric and family-centered care. To reduce the risk of bias, however, the investigator used a reflexive research approach during the data collection and analysis. Additionally, all emerging interpretations were

thoroughly discussed with a senior qualitative researcher to ensure an appropriate level of analytical rigor.

Ethical considerations

Ethical approval was sought and received from the Jordanian Royal Medical Services in Amman, Jordan prior to data collection. Participation was voluntary, and parents who did not volunteer to participate were reassured of the lack of any consequence. All participants were guaranteed anonymity and privacy; to assist with this each participant was assigned a pseudonym and numerical identifier. The informed consent form and information sheet both clearly noted that the identity of each participant was to be protected at all times.

RESULTS

Participant characteristics

The study was conducted on 20 mothers whose children had cancer. Two of the children were adolescents, five were school-age, two were preschoolers, nine were toddlers, and two were infants. The sample featured mothers of an equal number of male and female children with cancer. The most common cancer diagnosis was leukemia (Table 1). Table 2 offers more details of participant characteristics.

Transformative coping journeys

This theme is based on the personal reflections of mothers of children with cancer. Many of them report gradually beginning to see life in a new light, acquiring new strengths within themselves and drawing valuable lessons from their experiences. All of the subthemes identified within this category can be attributed to adjustment by these mothers to a new reality, which helps them make sense of what has happened and transforms their future expectations.

Shifting priorities: Twelve mothers ($n = 12$) adjusted their priorities according to their beliefs about what would be appropriate for their children under their current circumstances. While they initially sought all possible cures to treat their ailing children, this goal evolved with time, particularly for those who closely observe their children enduring radical treatment: For example, the mother of Rami noted that she and her husband had declined intensive chemotherapy treatment that they thought would bring him more pain and only slightly improve his health. Mothers thus came to balance the costs and the benefits of any form of treatment before making decisions to either continue or stop such efforts. This approach is applicable in a number of life situations and undertakings, and mothers reported paying moving attention from their long-term perspectives for their children to identifying short-term programs to improve their quality of life and pleasure: Since he got the disease, he does not like education anymore, he wants to play all the time, he wants to use all his energy, he wants to do many things, he likes to ride an electric bicycle. This is all right with us provided that it mitigates his pain (Walid's mother).

Holding back tears to appear strong: Ten mothers ($n = 10$) spoke of demonstrating resilience in response to their children's suffering, viewing this as a crucial tactic for confronting such crises. Recognizing the impact of their emotional states on their children's ability to cope with illness, they concealed their own discomfort or distress and resolved to lead ordinary, normal lives to help their children stay happy and manage their distress: Look at his reaction ...when he sees me happy...smiling...taking care of myself...living my life well...he is happy and relaxed. When he sees my tears...he becomes tense...tired...and asks, "What [did] the doctors say?" ... [When] I say nothing else...he [becomes] worried and unhappy (Sanad's mother). I do not wish to cry in his presence...because when he sees me crying, he cries...I typically use the restroom to avoid him seeing me weep in front of him. I wash my face and return (Loai's mother).

Learning important lessons from experiences: Eight mothers ($n = 8$) spoke of learning valuable lessons from their children's treatment journeys. Interactions between the mothers led to sharing treatment information and comparisons of their children's prognoses. However, improvements in the conditions of other children with cancer occasionally negatively affected mothers, especially if their children's conditions worsened. Some mothers were thus significantly affected by information received from other mothers, especially at the beginning of their children's therapy, when they were eager to learn as much as possible about their children's health. However, several mothers mentioned learning not to listen to other mothers or to compare children's progress: So, the first lesson I learned is not to follow any mother...instead to ask nurses and doctors about the condition of my child...as every condition is different from each other (Yaser's mother).

Reconstructing futures while delaying personal dreams: Nine mothers ($n = 9$) mentioned that their children's illness had changed their future plans. Cancer was noted to dramatically interfere with day-to-day living as well as long-term strategies and visions, both of which negatively affected their identities and elicited feelings of helplessness. Temporal and financial losses also impaired their ability to plan and implement their future goals: This shows that we were so high in our dreams: Now we are down. Our lives have stagnated; nothing is happening, our dream is always thwarted. When we set out to transform our lives, fate intervenes as soon as Hatim relapses (Hatim's mother).

According to some mothers, their attention has been switched to making changes in their daily lives, particularly in terms of making changes that concern the state of their children: Originally, I had wanted to construct a house to Walid...this is because I wanted to do things in my children's lives...I wanted to bring it to them...I wanted to consider their future...but all this suddenly came to a halt...this is because I can no longer do [that].

Table 1 Children's characteristics

No.	Pseudonym	Gender	Age	Diagnosis
1	Rami	M	14 years	Brain tumor
2	Jihad	M	3 months	Leukemia
3	Sanad	M	13 years	Sarcoma
4	Hala	F	3 years	Leukemia
5	Ahmad	M	3 years	Leukemia
6	Yaser	M	5 years	Leukemia
7	Walid	M	11 years	Lymphoma
8	Hatim	M	2 years	Leukemia
9	Wesal	F	1.5 years	Leukemia
10	Rola	F	1.5 yrs	Leukemia
11	Dania	F	3 years	Leukemia
12	Rania	F	6 years	Leukemia
13	Zubaida	F	3 years	Lung cancer
14	Mariam	F	7 months	Leukemia
15	Mona	F	9 years	Leukemia
16	Walaa	F	5 years	Wilms cancer
17	Tamer	M	3 years	Osteosarcoma
18	Anas	M	11 years	Leukemia
19	Loai	M	12 years	Leukemia
20	Jomana	F	2.5 years	Renal cancer

M: Male; F: Female.

Challenges

Emotional irritability in the sick child: Nine mothers ($n = 9$) reported difficulties due to behavioral and emotional changes in their children such as irritability, nervousness, and regression following the onset of their disease that adversely impacted the capacity of their parents to look after them and made parenting more complicated. In one instance, Walid's mother complained that despite previously being an empathetic and sensitive child, his behavior changed and he started abusing his siblings, expecting all of his demands to be satisfied. He also became obstinate and expected to do anything he wished, no matter how other people felt.

In a similar way, Dania's mother said that her daughter had regressed emotionally: A month before she fell sick, I [trained her to use the toilet rather than a diaper] ... I took away the bottle... When she got ill, all these things came back (Dania's mother).

Impact on family life and daily routines: Fourteen mothers ($n = 14$) were distressed about the difficulty of managing their homes while staying with their hospitalized children; they found it particularly difficult to leave their other children behind without adequate support. Determining who could care for their healthy children was one of their primary stressors.

Eleven mothers ($n = 11$) described arranging and coordinating care for their other children during hospital stays as being challenging, particularly with respect to distributing children among relatives while maintaining family routines. One mother explained how she organized care in advance to ensure her children were looked after within the family network: Certainly...one day prior to hospital admission...I distribute my children...My other son is typically sent to my sister because she can care for him...The other daughters are sent to the home of my parents (Hatim's mother).

Relationships between parents were also notably affected by their children's illnesses: My husband...I take less care of social support [for] him than I did before her illness...He is identical to me. Our primary concern is for our daughter (Zubaida's mother).

Preparing the home environment for a child after hospitalization: Eight mothers ($n = 8$) reported challenges related to preparing their home environments for their children's return after hospitalization, with issues raised including modifying the physical environment, ensuring cleanliness, and addressing emotional, medical, and nutritional needs. The need for emotional preparation was an aspect that many mothers considered important to help their children cope with

Table 2 Characteristics of study participants

Characteristics	Number
Age group (years)	
20-30	7
31-40	10
More than 40	3
Relationship to child	
Mother	20
Marital status	
Married	20
Widow	0
Divorce	0
Literacy level	
Primary level	3
Secondary level	11
Diploma (2 years)	3
BS.C	3
Occupation	
Housewife	17
Employee	3

changes after chemotherapy treatment: She was fond of her hair, however on her arrival at this stage I reassured her and made her realize that hair loss was a temporary side effect to treatment. I talked to her in a relaxed manner and motivated her, telling her that her hair would grow back even prettier (Hala's mother).

Several examples demonstrated that parents modify and transform their homes to support their children's healing journeys.

Support systems

Emotional and practical support from extended family: Sixteen mothers ($n = 16$) reported that they received assistance from the women in their extended families such as mothers-in-law, sisters, and sisters-in-law. While some of their husbands also helped with domestic chores, long work hours prevented many of them from taking full responsibility for such needs. The need for assistance was also influenced by several additional factors, including the age and number of other children and the distance to their relatives' homes. For example, the mothers of Ahmad, Wesal, and Dania mentioned that their other children were old enough to be left at home alone for short periods, while the mothers of Jihad, Hala, Rola, Zubaida, Tamer, Anas, and Loai had to seek more assistance from their extended family. The forms of assistance requested included visiting the sick children to reassure them about their health and caring for the family's other children, as illustrated by the experience of Rami's mother quoted below. Overall, support from extended families allowed mothers to remain with their hospitalized children for longer periods, alleviating their feelings of guilt and stress: I feel there is compassion for the condition of our children. This implies that individuals support the child's family instead of being alone...when you are alone, you may break down from overthinking, but when someone comes to you, you feel much more relaxed...I left my son...the other one...with my mother-in-law...she helped me a lot (Rami's mother).

Belief in destiny and God's will: The mothers' faith in destiny was a key coping mechanism identified within this study to help them overcome the situations they faced. All of them believed that their children's illness was God's will. For instance, Walaa's mother believed that any illness is a test from God to evaluate patience, while Taim's mother reported that her belief in God helped her transition from grief to acceptance and to seek alleviation from suffering by reading the Quran: He [her husband] also accepted God's will and read the Quran to alleviate his suffering...I cried a lot, but I asked God for forgiveness..." Why me? Why does my child have this disease?" Then, when I calmed down, I said, "Thanks, God...he is still superior to others" (Taim's mother).

Despite the beneficial impact of these religious beliefs, which promoted acceptance of their children's conditions, such acceptance was not consistently sustained. There were times of doubt, particularly during periods of intense pain for their children, which highlight this as an unstable coping process. Nevertheless, the spiritual beliefs of the mothers help reframe their suffering, fostering resilience and decreasing emotional pain.

DISCUSSION

This study provides valuable contextual information regarding the life experiences of Jordanian mothers of children with cancer, a relatively underrepresented group across pediatric oncology qualitative studies. This study thus expands the current literature by demonstrating the influence of the cultural expectations, extended families, and spiritual beliefs on mother's ability to cope, caregiving practices, and support-seeking behaviors, highlighting the importance of these factors in non-high-income contexts. The results further emphasize the need to use culturally responsive family-centered psychosocial interventions in pediatric oncology services in this region. Further studies are, however, indicated to develop understanding of the views of the fathers in these cases, as well as to examine longitudinal caregiving experiences and the differences made by children having different types of cancer and being at different stages to guide the development of more comprehensive support models.

Compared to the results from high-income states, mothers in Jordan demonstrated similar levels of emotional distress, family life disruption, and pressure to address caregiving needs, including those for their other children. Unlike most high-income environments, which have easily accessible formal psychosocial and support services, in Jordan, mothers were more likely to use their extended families, community support, and their spiritual beliefs to assist them in dealing with their children's illnesses. Studies in other low- and middle-income countries that may lack formal support systems, such as Ghana and Nigeria, have reported similar dependence on informal social and spiritual support among the family caregivers of cancer patients[12,13]. Family and community resources were highlighted particularly as significant by the caregivers in Kenya, as they needed to cope with these caregiving problems in a scenario with highly limited formal services. Such findings illustrate both how socioeconomic background and healthcare services influence experiences of caregiving and the similarities that exist in terms of maternal concerns across varying global locations.

The study's findings uncovered three major themes, with several subthemes, providing valuable insights into the lived experiences of Jordanian mothers of children with cancer. They also reveal an interesting homogeneity of response. The mothers in this research came from relatively similar social backgrounds. They were all married, most of them were housewives, and most had finished high school. Although they were all caring for children with differing kinds of cancer, they faced their challenges in very similar ways, suggesting that the expectations of Jordanian mothers and women in general may profoundly impact how they deal with their children's conditions. Concepts of shared social duties in particular may help families deal with the hardships of caregiving regardless of children's medical conditions.

While the most common diagnosis for the children in this study was leukemia, mothers of children with other cancer types reported similar experiences in terms of emotional load, caregiving challenges, reliance on relatives, and spiritual support. Variation was most evident based on the intensity of treatment and the course of the disease rather than cancer type itself. The parents of children who relapsed or underwent long-term treatment reported increased uncertainty, emotional exhaustion, and disruption of future plans in almost all cases. While the qualitative nature and the sample size of this research do not permit a close comparison across all stages of cancer or diagnoses, these findings indicate the potential for common issues related to caregiving that are not specific to particular types of cancer to be identified. More studies using a wider range of samples should thus be conducted to develop understanding of how maternal experiences may be influenced by children's cancer types and stages over time.

The findings reveal the ways in which mothers adapt to their children's illnesses by shifting their priorities, seeking to appear stronger, learning from their experiences, and delaying their dreams. Caring for children with cancer causes them to alter their schedules and priorities, develop understanding of the futility of comparing their children to others, and learn the importance of asking healthcare professionals about their children's conditions.

The findings also reveal the importance of the needs of the home during hospitalization, with mothers expressing significant concerns about finding responsible individuals to care for their other children during any hospital stays. As noted by Concha Méndez *et al*[14], hospitalization is associated with physical distancing, which significantly affects family routines, causing some parents to experience difficulties in caring for both the sick child and their other children at home simultaneously. Mothers are expected to develop an almost doubled presence, providing special care for the child with cancer in addition to caring for their other children effectively.

Previous studies have similarly reported hospitalization as substantially impacting the family environment and relationships at home. Families may be disrupted by both changing roles and physical distancing[15], with any healthy children at home potentially feeling abandoned by their parents during the sick child's hospitalization period[16,17].

In this study, preparing the home environment for a child's return after hospitalization also emerged as an important subtheme. The experience of caring for children with cancer thus enabled mothers to gain new knowledge about nutritional needs, food preparation, and cleanliness, to the benefit of their children. As in previous studies, mothers reported commonly learning new useful information and care skills in the hospital-home transition period regarding topics such as infection prevention schedules, hygiene activities, nutrition and food safety, and treatment-related care activities, all of which are seen as necessary to maintain their children's safety at home[3,18]. These changes are also, however, indicative of the increasing responsibility and burden of care placed on mothers, who pay close attention to the health of their children during the process of switching from hospital-based care to home-based care.

The findings of this study also revealed the importance of social support systems for parents, which are primarily provided by extended family members in this region; this aligns with findings from previous studies[3,18-20]. In contrast, Shattnawi *et al*[8] indicated that fathers assumed responsibilities for caregiving and household tasks, duties they may not have previously embraced within Jordanian culture. This disparity in findings may be attributed to the restricted sample size in the current case, which complicates generalization even within a single culture. Furthermore, in Jordanian culture, while the extended family is expected to help the nuclear family unit in stressful situations, this is conditional upon several elements, including ability to help, the availability of time, kinship bonds, and the nature of various familial relationships.

All of the mothers in the current study did state that they received spousal support, which might serve as an aid to reduce emotional distress and help them remain in their caring roles. Past studies have indicated that marital status and spousal support are both linked to the level of caregiver burden and quality of life in family caregivers for cancer patients: Single caregivers tend to report higher caregiver burdens and reduced quality of life[21]. In this study, most participants were married, and many of them were thus able to take temporary leave or to work fewer hours so as to give care; this suggests that employment status may also affect the strain of giving care. Working caregivers were observed to be under increased caregiving burden, implying extra pressure on those who have to balance paid employment and caregiving duties[22]. It is reasonable to assume that single mothers or mothers with no spousal support would thus be burdened even more, in terms of both emotional stress and facing greater logistical difficulties, and such pressures are likely to impact their personal goals and coping mechanisms. These considerations imply that future studies are required to examine the ways in which marital and employment status intersect in terms of support and coping among various groups of caregivers.

The current study did not report any significant changes in support over time. Adu-Assiamah[19], however, revealed that family support decreased over time across several factors, including reductions in the amount of time spent with friends and other family members. This suggests that it is important to investigate how social support might change over time, especially where children are hospitalized over extensive periods.

The mothers in the current study commonly adopted religious or spiritual coping mechanisms to deal with the stress caused by their children's illnesses. Spiritual coping refers to the act of employing cognitive and behavioral techniques rooted in personal beliefs and values to manage stress[23]. Pargament's Theory of Religious/Spiritual Coping elucidates how individuals utilize religion or spirituality to comprehend and cope with significant life challenges, including disease, trauma, or bereavement[9]. The religious faith of the mothers in the current study must thus be seen not merely as a belief system but as a dynamic coping mechanism offering meaning, control, comfort, closeness, and transformation, allowing them to better endure their children's illnesses.

The mothers in this study believed that their children's illnesses were destiny or God's will. Similarly, Atout[24] reported that respondents' spiritual beliefs help them accept their children's illnesses, develop patience, and remain engaged despite the deep pain and suffering experienced by their children. Arabiat *et al*[7] also reported that Jordanian mothers believed that their children's cancer was caused by both supernatural and biomedical explanations, allowing them to assign meaning to their children's illnesses. While mothers in this study revealed that their religious beliefs helped them accept their children's condition, this acceptance was not constant, however, with fluctuations particularly evident when they experienced extreme stress.

The mothers' coping processes in this work could be explained by Folkman[6] Transactional Model of Stress and Coping, according to which, mothers may regard their children's cancer diagnoses as a form of life-threatening peril for both the child and the family more widely. During their main appraisal, they may thus assess the distress caused by chemotherapy treatment as a substantial source of harm. During secondary appraisal, they may then evaluate their resources, such as spiritual belief, faith in God, support from extended family, and prior experiences, and seek to manage the perceived threat. When mothers are deprived of these resources, they may experience emotional turmoil and spiritual challenges, including seeking to conceal their vulnerabilities from their children and seeing their children's illnesses as divine trials.

Study limitations

This study has several limitations. The qualitative approach used makes it difficult to generalize the results of this study to other contexts, as maternal experiences from various cultures and nationalities are likely to differ from those reported by Jordanian mothers. Furthermore, the limited data collection period made it difficult to recruit mothers with varying demographic characteristics, limiting generalizability even in similar settings. Additionally, no fathers were interviewed for this study, as mothers were more likely to be present during their children's hospitalization.

CONCLUSION

Health professionals can play various important roles in helping parents care for both themselves and their children and wider families due to their prolonged, intimate interactions with such people[25]. The need for flexible, relational psychosocial support services grounded in the daily lives of parents[25], which recognizes and addresses the distress and personal difficulties associated with parenting a critically ill child and encompasses interparental communication and comprehension, in addition to self-awareness and self-care, is clear within the results of this study. Further implications regarding the necessity of enhancing health professionals' knowledge and education regarding the repercussions of children's cancer on families, including mitigating stigma, providing a more nurturing atmosphere for parents, and enhancing family-centered education and psychosocial support within healthcare settings are thus inherent in this result.

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